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 / Practice Exam - 1

Correct Answer

Next Q

Practice Exam - 1

Question 110 of 117



A 72-year-old male with longstanding history of obesity (BMI 42 Kg/m²), DM, and HTN presents to clinic for evaluation for worsening DOE and abdominal bloating. On examination, he has no LE edema or ascites. JVP is estimated at 6 cm H₂O. EKG reveals NSR at 78 bpm with a nonspecific IVCD. CXR is unremarkable. TTE revealed a LVEF of 50%, normal RV function, PASP of 48 mmHg and grade 3 diastolic dysfunction with an E/e' of 16. Blood work is notable for Na 137 mmol/L, K 4.5 mEq/L, BUN 28 mg/dl, Cr 1.4 mg/dl. CBC is unremarkable. NT proBNP is 432 pg/ml. TSH is within normal limits.

He is diagnosed with HFpEF and initiated on empagliflozin 10 mg daily. He feels better but continues to report dyspnea and abdominal bloating. NT proBNP remains elevated. Stress testing performed with PET CT was negative for ischemia. Vitals remain stable with a BP of BP 128/83 mmHg, HR 84 bpm.

Which of the following should be the next step in his treatment?

- A. Initiate spironolactone 25 mg
- ☒ B. Initiate furosemide 20 mg daily
- C. Initiate sacubitril-valsartan 24-26 mg twice daily
- D. Initiate isosorbide mononitrate 30 mg daily

Commentary References

Testing Point: The board-certified AHFTC specialist should know the data supporting guideline-based therapies in HFpEF

Correct Answer: B. Initiate furosemide 20 mg daily

Rationale: The patient has a diagnosis of HFpEF with some improvement after initiation of empagliflozin however he remains volume overloaded based on symptoms and natriuretic peptides. As such, the appropriate next step would be decongestion with loop diuretic (Answer B).

Initiating the patient on spironolactone 25 mg (Answer A) would be reasonable after initiation of loop diuretics however would not be the correct next step nor is the subsequent reasoning of answer A correct. The TOPCAT trial of spironolactone in HFpEF did not show statistically significant improvement in the composite endpoint of death, aborted cardiac arrest or HF hospitalization between the spironolactone and placebo groups. However, it is imperative to note that post hoc analyses showed differential effects in the Americas as compared to Russia–Georgia with potential benefits seen in the Americas. A separate posthoc analysis also showed an interaction between ejection fraction and spironolactone with the greatest benefit seen with LVEF less than 50%.

Answer C: The initiation of an ARNi therapy (sacubitril-valsartan) is also reasonable in HFpEF however the PARAGON trial comparing sacubitril-valsartan and placebo did not achieve a statistically significant difference in its primary composite outcome of cardiovascular death or HF hospitalization. Exploratory post hoc analyses did show that sacubitril-valsartan appeared to have a greater benefit in patients with lower ejection fractions and in women more than men.

Answer D: Lastly, the NEAT-HFpEF (Nitrate’s Effect on Activity Tolerance in Heart Failure With Preserved Ejection Fraction) trial evaluating isosorbide mononitrate use in HFpEF showed no benefit and potentially worse exercise tolerance in HFpEF patients treated with nitrates.

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